Page 1: Healthcare Insurance Operations Manual

Purpose

This fictional dataset is designed for VaultRAG testing. It contains operational rules for eligibility, pre-authorization, claims, reimbursement, denials, appeals, billing, and security.

Policy ID

HIO-2026-TEST-01. Effective 1 September 2026.

General Rule

Operational decisions must consider the member plan, service date, provider contract, and documentation available in the claim record.

Member Eligibility

Coverage is available only when the policy is active on the date of service. Services after termination require verification.

Test Notice

This document is fictional and must not be used for real healthcare, insurance, reimbursement, or medical decisions.

Page 2: Member Eligibility and Coverage

Eligibility Verification

Verify member ID, policy status, effective date, termination date if applicable, and plan before adjudication.

Coverage Status

ACTIVE means coverage is available subject to plan terms. TERMINATED means services after termination are not eligible. PENDING REVIEW requires manual verification.

Waiting Period

The fictional Standard Plus plan has a 30-day waiting period for elective inpatient services beginning on the policy effective date. Emergency services are exempt from this waiting period.

Dependent Coverage

A dependent must be listed on the policy before a claim is processed under the dependent's member ID. Name mismatches require verification.

Coordination of Benefits

When another insurer may be responsible for the same service, the claim is flagged for coordination-of-benefits review before final payment.

Page 3: Pre-Authorization Rules

Required Authorization

Pre-authorization is required for planned inpatient admissions, selected advanced imaging, elective surgery, and home infusion under Standard Plus.

Timing

Planned authorization requests should be submitted at least 3 business days before the scheduled service.

Urgent Requests

Urgent requests may be submitted on the day the need is identified and must contain sufficient clinical and administrative information.

Authorization Number

An approved request receives an authorization number, which must be associated with the later claim.

No Authorization

If authorization is required and no valid authorization exists, route the claim to manual review rather than automatically paying it.

Change in Service

If the actual service materially differs from the authorized service, an updated authorization must be requested when required.

Page 4: Claim Submission Requirements

Standard Claim Package

A standard claim should contain the claim form, hospital or professional bill, itemized treatment information, payment receipt when applicable, discharge summary for inpatient care, and relevant supporting reports.

Electronic Claims

Electronic claims must include member ID, provider ID, service date, procedure information, diagnosis information where applicable, billed amount, and claim reference information.

Missing Documents

A claim missing required documentation is placed in DOCUMENTATION_PENDING status and the missing items are recorded.

Duplicate Claims

Claims with the same member, provider, service date, service category, and materially matching amount are checked for possible duplication.

Timely Filing

The fictional Standard Plus plan requires claims within 180 calendar days after the service date unless a documented exception applies.

Page 5: Inpatient Claims

Admission

An inpatient claim must include admission date, discharge date, facility information, member information, and applicable authorization details.

Discharge Documentation

The discharge summary should describe the admission outcome and relevant services. Missing documentation can place the claim into review.

Room and Board

Room and board is reimbursable only when the admission is covered and the billed stay is supported by the claim record.

Length of Stay

If billed length of stay differs from the authorized period, the difference must be reviewed before payment.

Readmission

A readmission within 7 calendar days of discharge is flagged for review. The flag alone does not determine payment or denial.

Page 6: Outpatient and Diagnostic Services

Advanced Imaging

MRI and CT services requiring authorization must have valid authorization before final payment.

Laboratory Services

Laboratory claims must identify service date, ordering provider where required, performing provider, procedure information, and billed amount.

Outpatient Procedures

An outpatient procedure must be checked against plan benefits and any authorization requirement.

Multiple Services

Multiple services on one date are evaluated under applicable rules. Bundling or duplicate-service edits may apply.

Invalid Provider

A claim with an invalid or inactive provider identifier is routed to provider-data review rather than automatically paid.

Page 7: Reimbursement and Payment Rules

Allowed Amount

The allowed amount is the amount recognized for reimbursement under the plan and provider agreement and may differ from the billed amount.

Member Responsibility

Copayment, deductible, and coinsurance are calculated according to plan benefits and accumulated responsibility.

Example Calculation

Illustrative example: allowed amount 1,000 units, deductible remaining 200 units, and 20% coinsurance after deductible. Member responsibility is 360 units and plan payment is 640 units.

Out-of-Network

Out-of-network reimbursement follows plan rules and may use a different allowed amount or member responsibility.

Payment Release

Payment release requires completion of eligibility, authorization, documentation, benefit, and duplicate checks.

Page 8: Denials and Exceptions

Denial Categories

Common fictional categories include inactive coverage, excluded service, missing authorization, timely filing failure, duplicate claim, invalid provider information, and insufficient documentation.

Missing Authorization

A missing-authorization denial identifies the affected service or claim line and references the applicable authorization requirement.

Excluded Service

A service excluded by the plan is not payable unless an applicable exception is documented.

Timely Filing

A claim after the filing deadline may be denied unless a documented exception applies.

Manual Review

Manual review is required when available information is insufficient to determine whether an automated rule should be applied.

Denial Notice

A denial notice should state the affected claim or service, general reason, and available review or appeal route.

Page 9: Appeals and Reconsideration

Appeal Window

For this fictional policy, reconsideration may be requested within 60 calendar days from the denial notice.

Required Information

An appeal should include claim/reference number, member ID, reason for disagreement, and supporting documentation.

Reconsideration

Review includes the original claim, applied rules, submitted evidence, and new documentation.

Additional Evidence

New documents may change the review outcome when they address the original denial reason.

Outcome Record

The appeal record must contain review date, reviewer or review team, decision, and supporting reason.

Escalation

Cases requiring specialized review are escalated according to internal workflow and remain in review until a determination is recorded.

Page 10: Audit, Security, and Test Scenarios

Audit Logging

The system should record document uploads, document access, claim-processing actions, authorization changes, payment actions, and appeal decisions.

Access Control

Users should receive only the access required for their role. Claim and policy information must not be exposed to unauthorized users.

Data Correction

Corrections should be traceable. Original values should not be silently overwritten when an audit trail is required.

Scenario A

Active member, valid inpatient authorization, complete claim package. Ask about eligibility, authorization, and inpatient requirements.

Scenario B

MRI claim without authorization number. Ask whether payment can proceed and what workflow applies.

Scenario C

Claim arrives 210 days after service. Ask about timely filing and denial handling.

Scenario D

Inpatient claim lacks discharge summary. Ask what status and documentation rule applies.

Scenario E

Invalid provider ID. Ask which review process applies.

Scenario F

Member appeals 45 days after denial. Ask whether the appeal is within the fictional window and what information is required.

Scenario G

Billed service materially differs from authorization. Ask what action is required.

Scenario H

Potential duplicate with same member, provider, service date, category, and amount. Ask what check applies.